

ANNEX I

SUMMARY OF PRODUCT CHARACTERISTICS

[NOTE: the following are those items of information required by Article 11 of Directive 2001/83/EC and current practice in the centralised procedure. In the case of advanced therapy medicinal products, these items are listed in Annex II of Regulation (EC) 1394/2007. A specific “[QRD template for advanced therapy medicinal products containing genetically modified cells](#)” is available.

For the full information to be included in each section, please refer to the “[Guideline on Summary of Product Characteristics](#)” as published on the website of the European Commission in the Notice to Applicants, Volume 2C. This guideline should be read in conjunction with other relevant guidance documents that can be found on the European Medicines Agency website, under “[Product information requirements](#)” (e.g. “[QRD Convention to be followed for the EMA-QRD templates](#)”).]

[The use of combined SmPCs for different strengths of the same pharmaceutical form is encouraged (for evaluation and after the adoption of the opinion for all languages) when the SmPCs are completely identical, except for the few strength-specific details (e.g. if the indications are different for the different strengths, the SmPCs cannot be combined). In case of combined terms, only the primary pharmaceutical form should be considered, e.g. “solution for injection in vial” and “solution for injection in pre-filled syringe” can be combined. No justification will be required, provided the above conditions are met. See “[Policy on combined Summaries of Product Characteristics \(SmPCs\)](#)” for full details of the process. For different strengths not meeting the criteria above (e.g. if the indications are different for the different strengths), applicants may present SmPCs for different strengths in one document for the evaluation process only, clearly indicating with titles the strength or presentation to which alternative text elements refer. However, a separate SmPC per strength and per pharmaceutical form, containing all pack-sizes related to the strength and pharmaceutical form concerned, will have to be provided as follows:

- English language version: immediately after adoption of the opinion.
- All other language versions: at the latest 25 days after adoption of the opinion (i.e. at the latest after incorporation of Member States comments).

105 See also: “[*The linguistic review process of product information in the centralised procedure*](#)”.]

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107 [Standard statements are given in the template to be used whenever they are applicable. If the applicant
108 needs to deviate from these statements to accommodate medicinal product-specific requirements,
109 alternative or additional statements will be considered on a case-by-case basis.]

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111 [Bracketing convention:
112 {text}: Information to be filled in
113 <text>: Text to be selected or deleted as appropriate
114 [green text]: Guidance and explanatory notes only; this text should not be included in the PI annexes.
115 (S)/(s): brackets to be deleted if term is in plural form, and brackets and ‘S’/‘s’ to be deleted if term is in
116 singular form.]

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[For medicinal products subject to additional monitoring ONLY:

The black symbol and the statements should only appear preceding section 1. The black symbol shall be a black inverted equilateral triangle; the symbol shall be proportional to the font size of the subsequent standardised text and in any case each side of the triangle shall have a minimum length of 5 mm. For the purpose of preparing the product information annexes, please use the black triangle as presented in this template (see below).]

<▼ This medicinal product is subject to additional monitoring. This will allow quick identification of new safety information. Healthcare professionals are asked to report any suspected adverse reactions. See section 4.8 for how to report adverse reactions.>

1. NAME OF THE MEDICINAL PRODUCT

[Guidance on the expression of strength is available in the “[ORD recommendations on the expression of strength in the name of centrally authorised human medicinal products \(as stated in section 1 of SmPC and in the name section of labelling and PL\)](#)”.]

{(Invented) name strength pharmaceutical form}

[No ® ™ symbols are to be included here and throughout the text; they can be reinstated in the printed materials. The units of the strength are to be presented as singular (e.g. milligram/mg rather than milligrams/mgs). Pharmaceutical forms such as “tablets” and “capsules” are to be written in plural.]

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

[Name of the active substance(s) in the language of the text.]

[For advanced therapy medicinal products ONLY:

Where an advanced therapy medicinal product contains cells or tissues, a detailed description of these cells or tissues and of their specific origin shall be provided, including the species of animal in cases of non-human origin. The following sub-headings shall be included:

<2.1 General description> [For advanced therapy medicinal products only]

<2.2 Qualitative and quantitative composition> [For advanced therapy medicinal products only, explanatory illustrations may be included, if necessary.]

<Excipient(s) with known effect>

<For the full list of excipients, see section 6.1.>

3. PHARMACEUTICAL FORM

[The pharmaceutical form must be presented in singular form according to the “[Standard terms](#)” published by the Council of Europe. If the patient-friendly term is used in the labelling, it must be included here in brackets after the full pharmaceutical form.]

<The score line is only to facilitate breaking for ease of swallowing and not to divide into equal doses.>

<The score line is not intended for breaking the tablet.>

<The tablet can be divided into equal doses.>

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

[Specify, if appropriate <This medicinal product is for diagnostic use only.>]

<{(Invented) name}> is indicated in <adults> <neonates> <infants> <children> <adolescents> <aged {x to y}> <years> <months>.>

4.2 Posology and method of administration

Posology

<Special populations>

[Additional sub-headings such as “Elderly”, “Hepatic impairment” or “Renal impairment” can be included if necessary.]

Paediatric population

<The <safety> <and> <efficacy> of {(Invented) name} in children aged {x to y} <months> <years> [or any other relevant subsets, e.g. weight, pubertal age, gender] <has> <have> not <yet> been established.> [One of the following statements should be added:

<No data are available.>

or <Currently available data are described in section <4.8> <5.1> <5.2> but no recommendation on a posology can be made.>]

<{(Invented) name}> should not be used in children aged {x to y} <years> <months> [or any other relevant subsets e.g. weight, pubertal age, gender] because of <safety> <efficacy> concern(s).> [concern(s) to be stated with cross-reference to sections detailing data (e.g. 4.8 or 5.1).]

<There is no relevant use of {(Invented) name} <in the paediatric population> <in children aged {x to y} <years> <months> [or any other relevant subsets, e.g. weight, pubertal age, gender] <for the indication of...>.> [specify indication(s).]

<{(Invented) name}> is contraindicated in children aged {x to y} <years> <months> [or any other relevant subsets, e.g. weight, pubertal age, gender] <for the indication of...> [specify indication(s).] (see section 4.3).>

Method of administration

{(Invented) name} is for {route of administration}.

<Precautions to be taken before handling or administering the medicinal product>

[Method of administration: directions for proper use by healthcare professionals or by the patient. Further practical details for the patient can be included in package leaflet, e.g. in the case of inhalers or subcutaneous self-injection. Explanatory illustrations may be included, if necessary, especially for advanced therapy medicinal products.]

<For instructions on <reconstitution> <dilution> of the medicinal product before administration, see section <6.6> <and> <12>.>

4.3 Contraindications

<Hypersensitivity to the active substance(s) or to any of the excipients listed in section 6.1 <or {name of the residue(s)}>.>

4.4 Special warnings and precautions for use

[For biological medicinal products, include the following statement:]

<Traceability

In order to improve the traceability of biological medicinal products, the name and the batch number of the administered product should be clearly recorded.>

[Sub-headings (e.g. “Interference with serological testing” “Hepatic impairment”, “QT prolongation”) should be used where necessary to facilitate readability (i.e. identification of information in lengthy section).]

<Paediatric population>

<Excipient(s) with known effect>

4.5 Interaction with other medicinal products and other forms of interaction

<No interaction studies have been performed.>

<Paediatric population>

<Interaction studies have only been performed in adults.>

4.6 Fertility, pregnancy and lactation

[For pregnancy and lactation statements, see [Appendix I.](#)]

[Additional sub-headings such as “Women of childbearing potential”, “Contraception in males and females” can be included, as appropriate.]

<Pregnancy>

<Breast-feeding>

<Fertility>

4.7 Effects on ability to drive and use machines

<{Invented) name} has <no or negligible influence> <minor influence> <moderate influence> <major influence> on the ability to drive and use machines.> [describe effects where applicable.]

<Not relevant.>

4.8 Undesirable effects

Summary of the safety profile

Tabulated list of adverse reactions

[For MedDRA frequency convention and system organ class database, see [Appendix II.](#)]

[Additional sub-headings should be used to facilitate identification of information on each selected adverse reaction and on each relevant special population, e.g.: “Description of selected adverse reactions” (alternatively the subsection could be named with the name of the relevant adverse reaction), “Other special populations”.]

<Paediatric population>

[For ALL medicinal products:

The following sub-heading and statements should appear at the end of section 4.8:]

Reporting of suspected adverse reactions

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via the national reporting system listed in [Appendix V](#).*

[*For the printed materials: No reference to Appendix V should be included in the printed materials. The above grey-shaded terms will only appear in the published version of the approved product information annexes on the European Medicines Agency website. The actual details of the national reporting system (as listed in Appendix V) of the concerned Member State(s) shall be displayed on the printed version. Linguistic adjustments may also be necessary depending on the grammatical rules of the languages used.]

4.9 Overdose

[Additional sub-headings, such as “Symptoms” or “Management” can be included, if necessary.]
<Paediatric population>

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: {group} [The therapeutic subgroup (i.e. 2nd level of the [WHO classification](#)) should be included, with the 3rd and/or 4th level being recommended], ATC code: <{code}><not yet assigned>

[For medicinal products authorised as similar biological medicinal products, include the following statement:]

<{(Invented) name} is a biosimilar medicinal product. Detailed information is available on the website of the European Medicines Agency <https://www.ema.europa.eu/en>.>

[Tabular presentation of clinical efficacy and safety information may be used.]

<Mechanism of action>

<Pharmacodynamic effects>

<Clinical efficacy and safety>

<Paediatric population>

[If the European Medicines Agency has waived or deferred a paediatric development, the information should be given as follows under a relevant subheading:]

[For waivers applying to all subsets:]

<The European Medicines Agency has waived the obligation to submit the results of studies with <{(Invented) name}> [or for generics: <the reference medicinal product containing {name of the active substance(s)}>] in all subsets of the paediatric population in {condition as per paediatric investigation plan (PIP) decision, for the granted indication} (see section 4.2 for information on paediatric use).>

[For deferrals applying to at least one subset:]

<The European Medicines Agency has deferred the obligation to submit the results of studies with <{(Invented) name}> [or for generics: <the reference medicinal product containing {name of the active substance(s)}>] in one or more subsets of the paediatric population in {condition as per paediatric investigation plan (PIP) decision, for the granted indication} (see section 4.2 for information on paediatric use).>

[For medicinal products approved under “conditional approval”, include the following statement:]

<This medicinal product has been authorised under a so-called ‘conditional approval’ scheme.

This means that further evidence on this medicinal product is awaited.

The European Medicines Agency will review new information on this medicinal product at least every year and this SmPC will be updated as necessary.>

[For medicinal products approved under “exceptional circumstances”, include the following statement:]

<This medicinal product has been authorised under ‘exceptional circumstances’.

This means that <due to the rarity of the disease> <for scientific reasons> <for ethical reasons> it has not been possible to obtain complete information on this medicinal product.

The European Medicines Agency will review any new information which may become available every year and this SmPC will be updated as necessary.>

[For generic medicinal products, if the reference medicinal product has been approved under “exceptional circumstances”, include the following statement:]

<The reference medicinal product containing {active substance} has been authorised under ‘exceptional circumstances’. This means that <due to the rarity of the disease><for scientific reasons><for ethical reasons> it has not been possible to obtain complete information on the reference medicinal product. The European Medicines Agency will review any new information which may become available every year and this SmPC will be updated as necessary accordingly to the reference medicinal product SmPC.>

5.2 Pharmacokinetic properties

<Absorption>

<Distribution>

<Biotransformation>

<Elimination>

<Linearity/non-linearity>

[Additional sub-heading(s), such as “Renal impairment”, “Hepatic impairment”, “Elderly”, “Paediatric population” or “Other special populations” (to be specified) should be used, where appropriate.]

<Pharmacokinetic/pharmacodynamic relationship(s)>

5.3 Preclinical safety data

[Additional sub-headings, such as “Juvenile animals studies” can be included when necessary.]

<Non-clinical data reveal no special hazard for humans based on conventional studies of safety pharmacology, repeated dose toxicity, genotoxicity, carcinogenic potential, toxicity to reproduction and development.>

<Effects in non-clinical studies were observed only at exposures considered sufficiently in excess of the maximum human exposure indicating little relevance to clinical use.>

<Adverse reactions not observed in clinical studies, but seen in animals at exposure levels similar to clinical exposure levels and with possible relevance to clinical use were as follows:>

<Environmental risk assessment (ERA)>

[Refer to the “[Guideline on the environmental risk assessment of medicinal products for human use](#)”.]

6. PHARMACEUTICAL PARTICULARS

6.1 List of excipients

[Name of the excipient(s) in the language of the text according to the European Pharmacopoeia and listed in separate lines.]

[For advanced therapy medicinal products, preservative systems should be described.]

<None.>

6.2 Incompatibilities

<Not applicable.> [e.g. for solid oral pharmaceutical forms.]

<In the absence of compatibility studies, this medicinal product must not be mixed with other medicinal products.> [e.g. for parenterals.]

<This medicinal product must not be mixed with other medicinal products except those mentioned in section <6.6> <and> <12>.>

6.3 Shelf life

[Information on the finished product shelf life and on the in-use stability after first opening and/or reconstitution/dilution should appear here. Only one overall shelf life for the finished product is to be given even if different components of the medicinal product may have a different shelf life (e.g. powder and solvent). Full years must be stated as such and not in months (e.g. 2 years rather than 24 months).]

<...> <6 months> <...> <1 year> <18 months> <2 years> <30 months> <3 years> <...>

6.4 Special precautions for storage

[For storage condition statements, see [Appendix III](#).]

[General storage conditions of the finished medicinal product should appear here, together with a cross-reference to section 6.3 where appropriate:]

<For storage conditions after <reconstitution><dilution><first opening> of the medicinal product, see section 6.3.>

6.5 Nature and contents of container <and special equipment for use, administration or implantation>

[The optional part of the heading “and special equipment for use, administration or implantation” is for advanced therapy medicinal products only. In such a case, explanatory illustrations may be included, if necessary.]

[Multipack presentations should also be listed in this section, e.g. “multipacks containing 180 (2 packs of 90) film-coated tablets”.]

<Not all pack sizes may be marketed.>

6.6 Special precautions for disposal <and other handling>

[Include practical instructions for preparation and handling of the medicinal product, where applicable, including disposal of the medicinal product, and waste materials derived from the used medicinal product. Presentation of practical information using pictograms in addition to text may be considered, if necessary.]

<Use in the paediatric population>

<No special requirements <for disposal>.>

446 <Any unused medicinal product or waste material should be disposed of in accordance with local
447 requirements.>
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450 7. MARKETING AUTHORISATION HOLDER

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452 [Town/city and country name in the language of the text.]

453 {Name and address}

454 <{tel}>

455 <{e-mail}>

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458 8. MARKETING AUTHORISATION NUMBER(S)

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460 [EU numbers can be listed individually in separate lines or grouped (e.g. EU/0/00/000/001-005).]

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463 9. DATE OF FIRST AUTHORISATION/RENEWAL OF THE AUTHORISATION

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465 [As per SmPC guideline, the date should be stated in the following format:]

466 <Date of first authorisation: {DD month YYYY}>

467 <Date of <latest> renewal: {DD month YYYY}>

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469 [For the initial authorisation, the date should correspond to the initial date of the Commission Decision on
470 the marketing authorisation of the medicinal product concerned. It should not reflect individual
471 strength/presentation approvals introduced via subsequent variations and/or extensions.

472 For the (conditional) renewal, the date should correspond to the actual date of the Commission Decision
473 on the (conditional) renewal of the marketing authorisation.]

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476 10. DATE OF REVISION OF THE TEXT

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478 [Item to be completed by the Marketing Authorisation Holder (MAH) at time of printing.

479 For type IA variations affecting the product information, the date of revision of the text should be the date
480 of implementation of the change by the MAH.

481 For type II variations listed in Article 23(1a)(a), the date of revision of the text should be the date of the
482 Commission Decision amending the marketing authorisation.

483 For type II variations not listed in Article 23(1a)(a), which follow a yearly timeframe for update of the
484 respective Commission Decision, the date of revision of the text should be the date of the adoption of the
485 positive CHMP opinion on the variation to the terms of the marketing authorisation. For more details,
486 please consult the post-authorisation Q&A guidance.]

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488 <{MM/YYYY}>

489 <{DD/MM/YYYY}>

490 <{DD month YYYY}>

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493 <11. DOSIMETRY> [For radiopharmaceutical products ONLY.]

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496 <12. INSTRUCTIONS FOR PREPARATION OF RADIOPHARMACEUTICALS> [For
497 radiopharmaceutical products ONLY.]

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499 <Any unused medicinal product or waste material should be disposed of in accordance with local
500 requirements.>

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[Include the following statement for ALL medicinal products:]

Detailed information on this medicinal product is available on the website of the European Medicines Agency <https://www.ema.europa.eu/en><, and on the website of {name of Member State Agency (link)}>.*

[*The last part of the statement is optional, and **it is only to be displayed on the final printed materials.** It will not be included in the product information annexes as applicants may choose to include it for one or more Member States but not for all of them.]

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